

COMMITTEE ON HEALTH & HUMAN SERVICES  
HOUSE OF REPRESENTATIVES AMENDMENTS TO H.B. 2408  
(Reference to printed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

[~~GREEN STRIKEOUT IN BRACKETS~~] indicates new text removed from statute or previously enacted session law.

[~~Green strikeout in brackets~~] indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

<<~~Green strikeout in carets~~>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Section 32-1601, Arizona Revised Statutes, is amended to  
3 read:

4 32-1601. Definitions

5 In this chapter, unless the context otherwise requires:

6 1. "Absolute discharge from the sentence" means completion of any  
7 sentence, including imprisonment, probation, parole, community supervision  
8 or any form of court supervision.

9 2. "Appropriate health care professional" means a licensed health  
10 care professional whose scope of practice, education, experience, training  
11 and accreditation are appropriate for the situation or condition of the  
12 patient who is the subject of a consultation or referral.

13 3. "Approval" means that a regulated training or educational  
14 program to prepare persons for licensure, certification or registration  
15 has met standards established by the board.

16 4. "Board" means the Arizona state board of nursing.

17 5. "Certified nurse midwife" means a registered nurse who:

18 (a) Is certified by the board.

19 (b) Has completed a nurse midwife education program THAT IS  
20 approved or recognized by the board and educational requirements THAT ARE  
21 prescribed by the board by rule.

22 (c) Holds a national certification as a certified nurse midwife  
23 from a national certifying body THAT IS recognized by the board.

24 (d) Has an expanded scope of practice in providing health care  
25 services for women from adolescence to beyond menopause, including  
26 antepartum, intrapartum, postpartum, reproductive, gynecologic and primary  
27 care, for normal newborns during the first twenty-eight days of life and

1 for men for the treatment of sexually transmitted diseases. The expanded  
2 scope of practice under this subdivision includes:

3 (i) Assessing patients, synthesizing and analyzing data and  
4 understanding and applying principles of health care at an advanced level.

5 (ii) Managing the physical and psychosocial health care of  
6 patients.

7 (iii) Analyzing multiple sources of data, identifying alternative  
8 possibilities as to the nature of a health care problem and selecting,  
9 implementing and evaluating appropriate treatment.

10 (iv) Making independent decisions in solving complex patient care  
11 problems.

12 (v) Diagnosing, performing diagnostic and therapeutic procedures  
13 and prescribing, administering and dispensing therapeutic measures,  
14 including legend drugs, medical devices and controlled substances, within  
15 the scope of the certified nurse midwife practice after meeting  
16 requirements established by the board.

17 (vi) Recognizing the limits of the ~~nurse's~~ CERTIFIED NURSE  
18 MIDWIFE'S knowledge and experience by consulting with or referring  
19 patients to other appropriate health care professionals if a situation or  
20 condition occurs that is beyond the knowledge and experience of the  
21 CERTIFIED nurse MIDWIFE or if the referral will protect the health and  
22 welfare of the patient.

23 (vii) Delegating to a medical assistant pursuant to section  
24 32-1456.

25 (viii) Performing additional acts that require education and  
26 training as prescribed by the board and that are recognized by the nursing  
27 profession as proper to be performed by a certified nurse midwife.

28 6. "Certified nursing assistant" means a person who is registered  
29 on the registry of nursing assistants pursuant to this chapter to provide  
30 or assist in delivering nursing or nursing-related services under the  
31 supervision and direction of a licensed nursing staff member. Certified  
32 nursing assistant does not include a person who:

33 (a) Is a licensed health care professional.

34 (b) Volunteers to provide nursing assistant services without  
35 monetary compensation.

36 (c) Is a licensed nursing assistant.

37 7. "Certified registered nurse" means a registered nurse who has  
38 been certified by a national nursing credentialing agency THAT IS  
39 recognized by the board.

40 8. "Certified registered nurse anesthetist" means a registered  
41 nurse who meets the requirements of section 32-1634.03 and who practices  
42 pursuant to the requirements of section 32-1634.04.

43 9. "Clinical nurse specialist" means a registered nurse who:

44 (a) Is certified by the board as a clinical nurse specialist.

45 (b) Holds a graduate degree with a major in nursing and completes  
46 educational requirements as prescribed by the board by rule.

1 (c) Is nationally certified as a clinical nurse specialist or, if  
2 certification is not available, provides proof of competence to the board.

3 (d) Has an expanded scope of practice based on advanced education  
4 in a clinical nursing specialty that includes:

5 (i) Assessing clients, synthesizing and analyzing data and  
6 understanding and applying nursing principles at an advanced level.

7 (ii) Managing directly and indirectly a client's physical and  
8 psychosocial health status.

9 (iii) Analyzing multiple sources of data, identifying alternative  
10 possibilities as to the nature of a health care problem and selecting  
11 appropriate nursing interventions.

12 (iv) Developing, planning and guiding programs of care for  
13 populations of patients.

14 (v) Making independent nursing decisions to solve complex client  
15 care problems.

16 (vi) Using research skills and acquiring and applying critical new  
17 knowledge and technologies to nursing practice.

18 (vii) Prescribing and dispensing durable medical equipment.

19 (viii) Consulting with or referring a client to other health care  
20 providers based on assessment of the client's health status and needs.

21 (ix) Facilitating collaboration with other disciplines to attain  
22 the desired client outcome across the continuum of care.

23 (x) Performing additional acts that require education and training  
24 as prescribed by the board and that are recognized by the nursing  
25 profession as proper to be performed by a clinical nurse specialist.

26 (xi) Prescribing, ordering and dispensing pharmacological agents  
27 THAT ARE subject to the requirements and limits specified in section  
28 32-1651.

29 10. "Conditional license" or "conditional approval" means a license  
30 or approval that specifies the conditions under which the regulated party  
31 is allowed to practice or to operate and that is prescribed by the board  
32 pursuant to section 32-1644 or 32-1663.

33 11. "Delegation" means transferring to a competent individual the  
34 authority to perform a selected nursing task in a designated situation in  
35 which the nurse making the delegation retains accountability for the  
36 delegation.

37 12. "Disciplinary action" means a regulatory sanction of a license,  
38 certificate or approval pursuant to this chapter in any combination of the  
39 following:

40 (a) A civil penalty for each violation of this chapter, not to  
41 exceed \$1,000 for each violation.

42 (b) Restitution made to an aggrieved party.

43 (c) A decree of censure.

44 (d) A conditional license or a conditional approval that ~~fixed~~  
45 FIXES a period and terms of probation.

46 (e) Limited licensure.

47 (f) Suspension of a license, a certificate or an approval.

- 1 (g) Voluntary surrender of a license, a certificate or an approval.  
2 (h) Revocation of a license, a certificate or an approval.  
3 13. "Health care institution" has the same meaning prescribed in  
4 section 36-401.  
5 14. "Licensed health aide" means a person who:  
6 (a) Is licensed pursuant to this chapter to provide or to assist in  
7 providing nursing-related services authorized pursuant to section 36-2939.  
8 (b) Is the parent, guardian or family member by affinity or  
9 consanguinity of the Arizona long-term care system member receiving  
10 services who may provide licensed health aide services only to that member  
11 and only consistent with that member's plan of care.  
12 (c) Has a scope of practice that is the same as a licensed nursing  
13 assistant and may also provide medication administration, tracheostomy  
14 care, enteral care and routine ventilator care and therapy and any other  
15 tasks approved by the board in rule.  
16 (d) Has supervision requirements that are the same as a certified  
17 nursing assistant.  
18 15. "Licensed nursing assistant" means a person who is licensed  
19 pursuant to this chapter to provide or assist in delivering nursing or  
20 nursing-related services under the supervision and direction of a licensed  
21 nursing staff member. Licensed nursing assistant does not include a  
22 person who:  
23 (a) Is a licensed health care professional.  
24 (b) Volunteers to provide nursing assistant services without  
25 monetary compensation.  
26 (c) Is a certified nursing assistant.  
27 16. "Licensee" means a person who is licensed pursuant to this  
28 chapter or in a party state as defined in section 32-1668.  
29 17. "Limited license" means a license that restricts the scope or  
30 setting of a licensee's practice.  
31 18. "Medication order" means a written or verbal communication  
32 given by a certified registered nurse anesthetist to a health care  
33 professional to administer a drug or medication, including controlled  
34 substances.  
35 19. "Practical nurse" means a person who holds a practical nurse  
36 license issued pursuant to this chapter or pursuant to a multistate  
37 compact privilege and who practices practical nursing ~~as defined in this~~  
38 ~~section.~~  
39 20. "Practical nursing" includes the following activities that are  
40 performed under the supervision of a physician or a registered nurse:  
41 (a) Contributing to the assessment of the health status of  
42 individuals and groups.  
43 (b) Participating in the development and modification of the  
44 strategy of care.  
45 (c) Implementing aspects of the strategy of care within the nurse's  
46 scope of practice.

1 (d) Maintaining safe and effective nursing care that is rendered  
2 directly or indirectly.

3 (e) Participating in the evaluation of responses to interventions.

4 (f) Delegating nursing activities within the scope of practice of a  
5 practical nurse.

6 (g) Performing additional acts that require education and training  
7 as prescribed by the board and that are recognized by the nursing  
8 profession as proper to be performed by a practical nurse.

9 21. "Presence" means within the same health care institution or  
10 office as specified in section 32-1634.04, subsection A, ~~and~~ and available as  
11 necessary.

12 22. "Registered nurse" or "professional nurse" means a person who  
13 practices registered nursing and who holds a registered nurse license  
14 issued pursuant to this chapter or pursuant to a multistate compact  
15 privilege.

16 23. "Registered nurse practitioner" means a registered nurse who:

17 (a) Is certified by the board.

18 (b) Has completed a nurse practitioner education program **THAT IS**  
19 approved or recognized by the board and educational requirements **THAT ARE**  
20 prescribed by the board by rule.

21 (c) If applying for certification after July 1, 2004, holds  
22 national certification as a nurse practitioner from a national certifying  
23 body **THAT IS** recognized by the board.

24 (d) Has an expanded scope of practice within a specialty area that  
25 includes:

26 (i) Assessing clients, synthesizing and analyzing data and  
27 understanding and applying principles of health care at an advanced level.

28 (ii) Managing the physical and psychosocial health status of  
29 patients.

30 (iii) Analyzing multiple sources of data, identifying alternative  
31 possibilities as to the nature of a health care problem and selecting,  
32 implementing and evaluating appropriate treatment.

33 (iv) Making independent decisions in solving complex patient care  
34 problems.

35 (v) Diagnosing, performing diagnostic and therapeutic procedures,  
36 and prescribing, administering and dispensing therapeutic measures,  
37 including legend drugs, medical devices and controlled substances within  
38 the scope of registered nurse practitioner practice on meeting the  
39 requirements established by the board.

40 (vi) Recognizing the limits of the ~~nurse's~~ **REGISTERED NURSE**  
41 **PRACTITIONER'S** knowledge and experience by consulting with or referring  
42 patients to other appropriate health care professionals if a situation or  
43 condition occurs that is beyond the knowledge and experience of the  
44 **REGISTERED** nurse **PRACTITIONER** or if the referral will protect the health  
45 and welfare of the patient.

46 (vii) Delegating to a medical assistant pursuant to section  
47 32-1456.

1 (viii) Performing additional acts that require education and  
2 training as prescribed by the board and that are recognized by the nursing  
3 profession as proper to be performed by a REGISTERED nurse practitioner.

4 24. "Registered nursing" includes the following:

5 (a) Diagnosing and treating human responses to actual or potential  
6 health problems.

7 (b) Assisting individuals and groups to maintain or attain optimal  
8 health by implementing a strategy of care to accomplish defined goals and  
9 evaluating responses to care and treatment.

10 (c) Assessing the health status of individuals and groups.

11 (d) Establishing a nursing diagnosis.

12 (e) Establishing goals to meet identified health care needs.

13 (f) Prescribing nursing interventions to implement a strategy of  
14 care.

15 (g) Delegating nursing interventions to others who are qualified to  
16 do so.

17 (h) Providing for the maintenance of safe and effective nursing  
18 care that is rendered directly or indirectly.

19 (i) Evaluating responses to interventions.

20 (j) Teaching nursing knowledge and skills.

21 (k) Managing and supervising the practice of nursing.

22 (l) Consulting and coordinating with other health care  
23 professionals in the management of health care.

24 (m) Performing additional acts that require education and training  
25 as prescribed by the board and that are recognized by the nursing  
26 profession as proper to be performed by a registered nurse.

27 25. "Registry of nursing assistants" means the nursing assistants  
28 registry maintained by the board pursuant to the omnibus budget  
29 reconciliation act of 1987 (P.L. 100-203; 101 Stat. 1330), as amended by  
30 the medicare catastrophic coverage act of 1988 (P.L. 100-360; 102 Stat.  
31 683).

32 26. "Regulated party" means any person or entity that is licensed,  
33 certified, registered, recognized or approved pursuant to this chapter.

34 27. "Unprofessional conduct" includes the following, whether  
35 occurring in this state or elsewhere:

36 (a) Committing fraud or deceit in obtaining, attempting to obtain  
37 or renewing a license or a certificate issued pursuant to this chapter.

38 (b) Committing a felony, whether or not involving moral turpitude,  
39 or a misdemeanor involving moral turpitude. In either case, conviction by  
40 a court of competent jurisdiction or a plea of no contest is conclusive  
41 evidence of the commission.

42 (c) Aiding or abetting in a criminal abortion or attempting,  
43 agreeing or offering to procure or assist in a criminal abortion.

- 1 (d) COMMITTING any conduct or practice that is or might be harmful  
2 or dangerous to the health of a patient or the public.
- 3 (e) Being mentally incompetent or physically unsafe to a degree  
4 that is or might be harmful or dangerous to the health of a patient or the  
5 public.
- 6 (f) Having a license, certificate, permit or registration to  
7 practice a health care profession denied, suspended, conditioned, limited  
8 or revoked in another jurisdiction and not reinstated by that  
9 jurisdiction.
- 10 (g) Wilfully or repeatedly violating a provision of this chapter or  
11 a rule adopted pursuant to this chapter.
- 12 (h) Committing an act that deceives, defrauds or harms the public.
- 13 (i) Failing to comply with a stipulated agreement, consent  
14 agreement or board order.
- 15 (j) Violating this chapter or a rule that is adopted by the board  
16 pursuant to this chapter.
- 17 (k) Failing to report to the board any evidence that a registered  
18 ~~or~~ NURSE, practical nurse or ~~a~~ nursing assistant is or may be:
- 19 (i) Incompetent to practice.
- 20 (ii) Guilty of unprofessional conduct.
- 21 (iii) Mentally or physically unable to safely practice nursing or  
22 to perform nursing-related duties. A nurse who is providing therapeutic  
23 counseling for a nurse who is in a drug rehabilitation program is required  
24 to report that nurse only if the nurse providing therapeutic counseling  
25 has personal knowledge that patient safety is being jeopardized.
- 26 (l) Failing to self-report a conviction for a felony or  
27 undesignated offense within ten days after the conviction.
- 28 (m) Cheating or assisting another to cheat on a licensure or  
29 certification examination.
- 30 (n) ENGAGING IN SEXUAL CONDUCT WITH A CURRENT PATIENT OR WITH A  
31 FORMER PATIENT WITHIN SIX MONTHS AFTER THE LAST TREATMENT UNLESS THE  
32 PATIENT WAS THE LICENSEE'S SPOUSE AT THE TIME OF THE CONTACT OR,  
33 IMMEDIATELY PRECEDING THE NURSE-PATIENT RELATIONSHIP, WAS IN A DATING OR  
34 ENGAGEMENT RELATIONSHIP WITH THE LICENSEE. FOR THE PURPOSES OF THIS  
35 SUBDIVISION, "SEXUAL CONDUCT" INCLUDES:
- 36 (i) ENGAGING IN OR SOLICITING A SEXUAL RELATIONSHIP, WHETHER  
37 CONSENSUAL OR NONCONSENSUAL.
- 38 (ii) MAKING SEXUAL ADVANCES, REQUESTING SEXUAL FAVORS OR ENGAGING  
39 IN ANY OTHER VERBAL CONDUCT OR PHYSICAL CONTACT OF A SEXUAL NATURE.
- 40 (iii) INTENTIONALLY VIEWING A COMPLETELY OR PARTIALLY DISROBED  
41 PATIENT IN THE COURSE OF TREATMENT IF THE VIEWING IS NOT RELATED TO  
42 PATIENT DIAGNOSIS OR TREATMENT UNDER CURRENT PRACTICE STANDARDS.

1       Sec. 2. Section 32-1606, Arizona Revised Statutes, is amended to  
2 read:

3       32-1606. Powers and duties of board

4       A. The board may:

5       1. Adopt and revise rules necessary to carry into effect this  
6 chapter.

7       2. Publish advisory opinions regarding registered and practical  
8 nursing practice and nursing education.

9       3. Issue limited licenses or certificates if it determines that an  
10 applicant or licensee cannot function safely in a specific setting or  
11 within the full scope of practice.

12       4. Refer criminal violations of this chapter to the appropriate law  
13 enforcement agency.

14       5. Establish a confidential program for monitoring licensees who  
15 are chemically dependent and who enroll in rehabilitation programs that  
16 meet the criteria established by the board. The board may take further  
17 action if the licensee refuses to enter into a stipulated agreement or  
18 fails to comply with its terms. In order to protect the public health and  
19 safety, the confidentiality requirements of this paragraph do not apply if  
20 the licensee does not comply with the stipulated agreement.

21       6. On the applicant's or regulated party's request, establish a  
22 payment schedule with the applicant or regulated party.

23       7. Provide education regarding board functions.

24       8. Collect or assist in collecting workforce data.

25       9. Adopt rules to conduct pilot programs consistent with public  
26 safety for innovative applications in nursing practice, education and  
27 regulation.

28       10. Grant retirement status on request to retired nurses who are or  
29 were licensed under this chapter, who have no open complaint or  
30 investigation pending against them and who are not subject to discipline.

31       11. Accept and spend federal monies and private grants, gifts,  
32 contributions and devises to assist in carrying out the purposes of this  
33 chapter. These monies do not revert to the state general fund at the end  
34 of the fiscal year.

35       B. The board shall:

36       1. Approve regulated training and educational programs that meet  
37 the requirements of this chapter and rules adopted by the board.

38       2. By rule, establish approval and reapproval processes for nursing  
39 and nursing assistant training programs that meet the requirements of this  
40 chapter and board rules.

41       3. Prepare and maintain a list of approved nursing programs to  
42 prepare registered nurses and practical nurses whose graduates are  
43 eligible for licensing under this chapter as registered nurses or as  
44 practical nurses if they satisfy the other requirements of this chapter  
45 and board rules.

46       4. Examine qualified registered nurse and practical nurse  
47 applicants.



- 1           5. License and renew the licenses of qualified registered nurse and  
2 practical nurse applicants and licensed nursing assistants who are not  
3 qualified to be licensed by the executive director.
- 4           6. Adopt a seal, which the executive director shall keep.
- 5           7. Keep a record of all proceedings.
- 6           8. For proper cause, deny or rescind approval of a regulated  
7 training or educational program for failure to comply with this chapter or  
8 the rules of the board.
- 9           9. Adopt rules to approve credential evaluation services that  
10 evaluate the qualifications of applicants who graduated from an  
11 international nursing program.
- 12          10. Determine and administer appropriate disciplinary action  
13 against all regulated parties who are found guilty of violating this  
14 chapter or rules adopted by the board.
- 15          11. Perform functions necessary to carry out the requirements of  
16 the nursing assistant and nurse aide training and competency evaluation  
17 program as set forth in the omnibus budget reconciliation act of 1987  
18 (P.L. 100-203; 101 Stat. 1330), as amended by the medicare catastrophic  
19 coverage act of 1988 (P.L. 100-360; 102 Stat. 683). These functions shall  
20 include:
  - 21           (a) Testing and registering certified nursing assistants.
  - 22           (b) Testing and licensing licensed nursing assistants.
  - 23           (c) Maintaining a list of board-approved training programs.
  - 24           (d) Maintaining a registry of nursing assistants for all certified  
25 nursing assistants and licensed nursing assistants.
  - 26           (e) Assessing fees.
- 27          12. Adopt rules establishing acts that may be performed by a  
28 registered nurse practitioner or certified nurse midwife, except that the  
29 board does not have authority to decide scope of practice relating to  
30 abortion as defined in section 36-2151.
- 31          13. Adopt rules that prohibit registered nurse practitioners,  
32 clinical nurse specialists or certified nurse midwives from dispensing a  
33 schedule II controlled substance that is an opioid, except for an  
34 implantable device or an opioid that is for medication-assisted treatment  
35 for substance use disorders or as provided in section 32-3248.03.
- 36          14. Adopt rules establishing educational requirements to certify  
37 school nurses.
- 38          15. Publish copies of board rules and distribute these copies on  
39 request.
- 40          16. Require each applicant for initial licensure or certification  
41 to submit a full set of fingerprints to the board for the purpose of  
42 obtaining a state and federal criminal records check pursuant to section  
43 41-1750 and Public Law 92-544. The department of public safety may  
44 exchange this fingerprint data with the federal bureau of investigation.
- 45          17. Except for a licensee who has been convicted of a felony that  
46 has been designated a misdemeanor pursuant to section 13-604, revoke a  
47 license of a person, revoke the multistate licensure privilege of a person

1 pursuant to section 32-1669 or not issue a license or renewal to an  
2 applicant who has one or more felony convictions and who has not received  
3 an absolute discharge from the sentences for all felony convictions three  
4 or more years before the date of filing an application pursuant to this  
5 chapter.

6 18. Establish standards to approve and reapprove registered nurse  
7 practitioner and clinical nurse specialist programs and provide for  
8 surveys of registered nurse practitioner and clinical nurse specialist  
9 programs as the board deems necessary.

10 19. Provide the licensing authorities of health care institutions,  
11 facilities and homes with any information the board receives regarding  
12 practices that place a patient's health at risk.

13 20. Limit the multistate licensure privilege of any person who  
14 holds or applies for a license in this state pursuant to section 32-1668.

15 21. Adopt rules to establish competency standards for obtaining and  
16 maintaining a license.

17 22. Adopt rules to qualify and certify clinical nurse specialists.

18 23. Adopt rules to approve and reapprove refresher courses for  
19 nurses who are not currently practicing.

20 24. Maintain a list of approved medication assistant training  
21 programs.

22 25. Test and certify medication assistants.

23 26. Maintain a registry and disciplinary record of medication  
24 assistants who are certified pursuant to this chapter.

25 27. Adopt rules to establish the requirements for a clinical nurse  
26 specialist to prescribe and dispense drugs and devices consistent with  
27 section 32-1651 and within the clinical nurse specialist's population or  
28 disease focus.

29 28. Issue registrations to administer general anesthesia and  
30 sedation in dental offices and dental clinics pursuant to section 32-1272  
31 to certified registered nurse anesthetists who have national board  
32 certification in anesthesiology.

33 29. POST ALL WRITTEN SUBSTANTIVE POLICIES IN A CLEARLY IDENTIFIABLE  
34 SECTION ON THE BOARD'S PUBLIC WEBSITE.

35 C. The board may conduct an investigation on receipt of information  
36 that indicates that a person or regulated party may have violated this  
37 chapter or a rule adopted pursuant to this chapter. Following the  
38 investigation, the board may take disciplinary action pursuant to this  
39 chapter.

40 D. The board may limit, revoke or suspend the privilege of a nurse  
41 to practice in this state granted pursuant to section 32-1668.

42 E. Failure to comply with any final order of the board, including  
43 an order of censure or probation, is cause for suspension or revocation of  
44 a license or a certificate.

45 F. The president or a member of the board designated by the  
46 president may administer oaths in transacting the business of the board.

1 <<Sec. 3. Section 32-1644, Arizona Revised Statutes, is amended to  
2 read:

3 32-1644. Approval of nursing schools and nursing programs:  
4 application; maintenance of standards

5 A. The board shall approve all new prelicensure nursing,  
6 [REGISTERED] nurse practitioner and clinical nurse specialist programs  
7 pursuant to this section. A postsecondary educational institution or  
8 school in this state that is accredited by an accrediting agency  
9 recognized by the United States department of education desiring to  
10 conduct a registered nursing, practical nursing, [REGISTERED] nurse  
11 practitioner or clinical nurse specialist program shall apply to the board  
12 for approval and submit satisfactory proof that [+t] [THE INSTITUTION OR  
13 SCHOOL] is prepared to meet and maintain the minimum standards prescribed  
14 by this chapter and board rules.

15 B. The board or its authorized agent shall conduct a survey of the  
16 institution or program applying for approval and shall submit a written  
17 report of its findings to the board. If the board determines that the  
18 program meets the requirements prescribed in its rules, [+t] [THE BOARD]  
19 shall approve the applicant as either a registered nursing program,  
20 practical nursing program, [REGISTERED] nurse practitioner program or  
21 clinical nurse specialist program in a specialty area.

22 C. A nursing program approved by the board may also be accredited  
23 by a national nursing accrediting agency recognized by the board. If a  
24 prelicensure nursing program is accredited by a national nursing  
25 accrediting agency recognized by the board, the board does not have  
26 authority over [+t] [THE PROGRAM] unless any of the following occurs:

27 1. The board receives a complaint about the program relating to  
28 patient safety.

29 ~~2. The program falls below the standards prescribed by the board~~  
30 ~~in its rules.~~

31 ~~3.] [2.]~~ The program loses its accreditation by a national nursing  
32 accrediting agency recognized by the board.

33 ~~4.] [3.]~~ The program allows its accreditation by a national  
34 nursing accrediting agency recognized by the board to lapse.

35 D. From time to time the board, through its authorized employees or  
36 representatives, may resurvey all approved programs in [the] [THIS] state  
37 and shall file written reports of these resurveys with the board. If the  
38 board determines that an approved nursing program is not maintaining the  
39 required standards, [+t] [THE BOARD] shall [immediately] give written  
40 notice to the program specifying the defects. If the defects are not  
41 corrected within [a reasonable] [THE] time [as determined] [PRESCRIBED] by  
42 the board [BY RULE], the board may take either of the following actions:

43 1. Approve the program but restrict the program's ability to admit  
44 new students until the program complies with board standards.

45 2. Remove the program from the list of approved nursing programs  
46 until the program complies with board standards.

1 E. All approved nursing programs shall maintain accurate and  
2 current records showing in full the theoretical and practical courses  
3 given to each student.

4 F. The board does not have regulatory authority over the following  
5 approved [REGISTERED] nurse practitioner or clinical nurse specialist  
6 programs unless the conditions prescribed in subsection C [OF THIS  
7 SECTION] are met:

8 1. A [REGISTERED] nurse practitioner or clinical nurse specialist  
9 program that is part of a graduate program in nursing accredited by an  
10 agency recognized by the board if the program was surveyed as part of the  
11 graduate program accreditation.

12 2. A [REGISTERED] nurse practitioner or clinical nurse specialist  
13 program that is accredited by an agency recognized by the board.>>

14 <<Sec. 4. Section 32-1664, Arizona Revised Statutes, is amended to  
15 read:

16 32-1664. Investigation; hearing; notice

17 A. In connection with an investigation, the board or its duly  
18 authorized agents or employees may obtain any documents, reports, records,  
19 papers, books and materials, including hospital records, medical staff  
20 records and medical staff review committee records, or any other physical  
21 evidence that indicates that a person or regulated party may have violated  
22 this chapter or a rule adopted pursuant to this chapter:

23 1. By entering the premises, at any reasonable time, and inspecting  
24 and copying materials in the possession of a regulated party that relate  
25 to nursing competence, unprofessional conduct or the mental or physical  
26 ability of a licensee to safely practice nursing.

27 2. By issuing a subpoena under the board's seal to require the  
28 attendance and testimony of witnesses or to demand the production for  
29 examination or copying of documents or any other physical evidence.  
30 Within five days after a person is served with a subpoena, that person may  
31 petition the board to revoke, limit or modify the subpoena. The board  
32 shall do so if in its opinion the evidence required does not relate to  
33 unlawful practices covered by this chapter, is not relevant to the charge  
34 that is the subject matter of the hearing or investigation or does not  
35 describe with sufficient particularity the physical evidence whose  
36 production is required.

37 3. By submitting a written request for the information.

38 4. In the case of an applicant's or a regulated party's personal  
39 medical records, as defined in section 12-2291, by any means allowed by  
40 this section if the board either:

41 (a) Obtains from the applicant or regulated party, or the health  
42 care decision maker of the applicant or regulated party, a written  
43 authorization that satisfies the requirements of title 12, chapter 13,  
44 article 7.1.

1 (b) Reasonably believes that the records relate to information  
2 already in the board's possession regarding the competence, unprofessional  
3 conduct or mental or physical ability of the applicant or regulated party  
4 as it pertains to safe practice. If the board adopts a substantive policy  
5 statement pursuant to section 41-1091, it may authorize the executive  
6 director, or a designee in the absence of the executive director, to make  
7 the determination of reasonable belief.

8 B. A regulated party and a health care institution as defined in  
9 section 36-401 shall, and any other person may, report to the board any  
10 information the licensee, certificate holder, health care institution or  
11 individual may have that appears to show that a regulated party or  
12 applicant is, was or may be a threat to the public health or safety.

13 C. If a death or an incident requiring emergency medical response  
14 occurs in a dental office or dental clinic during the administration of or  
15 recovery from general anesthesia or sedation by a certified registered  
16 nurse anesthetist, the certified registered nurse anesthetist shall, and  
17 any other person may, report the death or incident to the board within  
18 seven business days after the occurrence.

19 D. The board retains jurisdiction to proceed with an investigation  
20 or a disciplinary proceeding against a regulated party whose license or  
21 certificate expired not more than five years before the board initiates  
22 the investigation.

23 E. Any regulated party, health care institution or other person  
24 that reports or provides information to the board in good faith is not  
25 subject to civil liability. If requested the board shall not disclose the  
26 name of the reporter unless the information is essential to proceedings  
27 conducted pursuant to this section.

28 F. Any regulated party or person who is subject to an investigation  
29 may obtain representation by counsel.

30 G. On determination of reasonable cause, the board, or if delegated  
31 by the board the executive director, may require a licensee, certificate  
32 holder or applicant to undergo at the expense of the licensee, certificate  
33 holder or applicant any combination of mental, physical or psychological  
34 examinations, assessments or skills evaluations necessary to determine the  
35 person's competence or ability to practice safely. These examinations may  
36 include bodily fluid testing and other examinations known to detect the  
37 presence of alcohol or drugs. If the executive director orders the  
38 licensee, applicant or certificate holder to undertake an examination,  
39 assessment or evaluation pursuant to this subsection, and the licensee,  
40 certificate holder or applicant fails to affirm to the board in writing  
41 within fifteen days after receipt of the notice of the order that the  
42 licensee, certificate holder or applicant intends to comply with the  
43 order, the executive director shall refer the matter to the board to allow  
44 the board to determine whether to issue an order pursuant to this  
45 subsection. At each regular meeting of the board the executive director  
46 shall report to the board data concerning orders issued by the executive

1 director pursuant to this subsection since the last regular meeting of the  
2 board and any other data requested by the board.

3 H. The board shall provide the investigative report if requested  
4 pursuant to section 32-3206.

5 I. If after completing its investigation the board finds that the  
6 information provided pursuant to this section is not of sufficient  
7 seriousness to merit disciplinary action against the regulated party or  
8 applicant, it may take either of the following actions:

9 1. Dismiss if in the opinion of the board the information is  
10 without merit.

11 2. File a letter of concern if in the opinion of the board there is  
12 insufficient evidence to support disciplinary action against the regulated  
13 party or applicant but sufficient evidence for the board to notify the  
14 regulated party or applicant of its concern.

15 J. Except as provided pursuant to section 32-1663, subsection F and  
16 subsection K of this section, if the investigation in the opinion of the  
17 board reveals reasonable grounds to support the charge, the regulated  
18 party is entitled to an administrative hearing pursuant to title 41,  
19 chapter 6, article 10. If notice of the hearing is served by certified  
20 mail, service is complete on the date the notice is placed in the mail.

21 K. A regulated party shall respond in writing to the board within  
22 thirty days after notice of the hearing is served as prescribed in  
23 subsection J of this section. The board may consider a regulated party's  
24 failure to respond within this time as an admission by default to the  
25 allegations stated in the complaint. The board may then take disciplinary  
26 actions allowed by this chapter without conducting a hearing.

27 L. An administrative law judge or a panel of board members may  
28 conduct hearings pursuant to this section.

29 M. In any matters pending before it, the board may issue subpoenas  
30 under its seal to compel the attendance of witnesses.

31 N. Patient records, including clinical records, medical reports,  
32 laboratory statements and reports, any file, film, other report or oral  
33 statement relating to diagnostic findings or treatment of patients, any  
34 information from which a patient or a patient's family might be identified  
35 or information received and records kept by the board as a result of the  
36 investigation procedure outlined in this chapter are not available to the  
37 public and are not subject to discovery in civil or criminal proceedings.

38 O. Hospital records, medical staff records, medical staff review  
39 committee records, testimony concerning these records and proceedings  
40 related to the creation of these records shall not be available to the  
41 public. They shall be kept confidential by the board and shall be subject  
42 to the same provisions concerning discovery and use in legal actions as  
43 are the original records in the possession and control of hospitals, their  
44 medical staffs and their medical staff review committees. The board shall  
45 use these records and testimony during the course of investigations and  
46 proceedings pursuant to this chapter.

1 P. If the regulated party is found to have committed an act of  
2 unprofessional conduct or to have violated this chapter or a rule adopted  
3 pursuant to this chapter, the board may take disciplinary action.

4 Q. The board may subsequently issue a denied license or certificate  
5 and may reissue a revoked or voluntarily surrendered license or  
6 certificate.

7 R. On application by the board to any superior court judge, a  
8 person who without just cause fails to comply with a subpoena issued  
9 pursuant to this section may be ordered by the judge to comply with the  
10 subpoena and punished by the court for failing to comply. Subpoenas shall  
11 be served by regular or certified mail or in the manner required by the  
12 Arizona rules of civil procedure.

13 S. The board may share investigative information that is  
14 confidential under subsections N and O of this section with other state,  
15 federal and international health care agencies and with state, federal and  
16 international law enforcement authorities if the recipient is subject to  
17 confidentiality requirements similar to those established by this section.  
18 A disclosure made by the board pursuant to this subsection is not a waiver  
19 of the confidentiality requirements established by this section.

20 [T. THE ARIZONA STATE BOARD OF NURSING SHALL TRANSFER, WITHIN FIVE  
21 BUSINESS DAYS, ANY COMPLAINT IT RECEIVES AGAINST A NURSING PROGRAM OR  
22 NURSING SCHOOL TO THE STATE BOARD FOR PRIVATE POSTSECONDARY EDUCATION, THE  
23 ARIZONA BOARD OF REGENTS OR THE COMMUNITY COLLEGE DISTRICT GOVERNING  
24 BOARD, AS APPLICABLE.]>>

25 Sec. 5. Title 32, chapter 15, article 3, Arizona Revised Statutes,  
26 is amended by adding sections 32-1664.01, 32-1664.02, 32-1664.03,  
27 32-1664.04 and 32-1664.05, to read:

28 32-1664.01. Investigation of complaints; confidentiality;  
29 limits; notice; evaluations; timeframe;  
30 definition

31 A. NOTWITHSTANDING ANY OTHER PROVISION OF THIS CHAPTER OR CHAPTER  
32 32 OF THIS TITLE TO THE CONTRARY:

33 1. EXCEPT AS SET FORTH IN PARAGRAPH 3 OF THIS SUBSECTION, THE BOARD  
34 SHALL REQUIRE COMPLAINANTS TO IDENTIFY THEMSELVES IN THE COMPLAINT AND  
35 MAKE THEMSELVES AVAILABLE FOR AN EVIDENTIARY INTERVIEW. COMPLAINANTS MAY  
36 REQUEST THAT THEIR IDENTITY REMAIN CONFIDENTIAL DURING THE INVESTIGATORY  
37 PROCESS. IF THE INVESTIGATORY PROCESS RESULTS IN A DETERMINATION THAT A  
38 VIOLATION OF LAW MAY HAVE OCCURRED, THE RESPONDENT IS ENTITLED TO REVIEW  
39 THE COMPLETE INVESTIGATORY FILE, INCLUDING THE IDENTITY OF THE COMPLAINANT  
40 EXCEPT AS PROVIDED IN PARAGRAPH 2 OF THIS SUBSECTION.

41 2. THE BOARD, ON FINDING CAUSE THAT A COMPLAINANT MAY REASONABLY  
42 FEAR RETALIATION OR BE ENDANGERED IF THE COMPLAINANT'S IDENTITY IS  
43 REVEALED OR IF THE COMPLAINT DIRECTLY IMPACTS PATIENT SAFETY, MAY CONTINUE  
44 TO MAINTAIN THE COMPLAINANT'S CONFIDENTIALITY FROM THE LICENSEE UNTIL THE  
45 CONCLUSION OF THE ADMINISTRATIVE PROCESS. THE BOARD MAY CONDUCT A CLOSED  
46 EVIDENTIARY HEARING IF THE COMPLAINANT REQUESTS THAT THE COMPLAINANT'S  
47 IDENTITY REMAIN PRIVATE AND THE BOARD HAS A REASONABLE BASIS TO CONDUCT

1 THE HEARING. A COMPLAINANT'S ANONYMITY MAY CONTINUE UNTIL EVIDENCE BY THE  
2 COMPLAINANT IS REQUIRED AT AN ADMINISTRATIVE PROCEEDING PURSUANT TO TITLE  
3 41 OR A LEGAL PROCEEDING.

4 3. THE BOARD MAY TAKE ACTION ON A COMPLAINT IF THE COMPLAINANT  
5 REFUSES TO IDENTIFY HIMSELF OR HERSELF ONLY IF THE BOARD HAS SUFFICIENT  
6 INFORMATION THAT A VIOLATION MAY HAVE OCCURRED WITHIN ITS JURISDICTION  
7 THAT DIRECTLY IMPACTS THE SAFETY OF PATIENTS WITHOUT THE TESTIMONY OF THE  
8 ANONYMOUS COMPLAINANT.

9 4. THE BOARD SHALL LIMIT AN INVESTIGATION OF A COMPLAINT TO THOSE  
10 INVESTIGATIVE SUBJECTS AND ACTIONS THAT ARE RELEVANT AND MATERIAL TO THE  
11 ISSUES RAISED IN THE COMPLAINT AND THAT WOULD REASONABLY BE TAKEN TO  
12 INVESTIGATE THE ISSUES IN THE COMPLAINT. THIS PARAGRAPH DOES NOT PROHIBIT  
13 THE BOARD FROM INVESTIGATING ANY ISSUE OR EVIDENCE OF UNPROFESSIONAL  
14 CONDUCT THAT IS DISCOVERED OR BROUGHT TO THE ATTENTION OF THE BOARD DURING  
15 THE INVESTIGATION OF THE ORIGINAL COMPLAINT.

16 5. ON REASONABLE BELIEF THAT A CRIME HAS BEEN COMMITTED, THE BOARD  
17 SHALL ~~[SEEK LEGAL ADVICE FROM ITS ASSIGNED LEGAL COUNSEL ON WHETHER THE~~  
18 ~~BOARD SHOULD]~~ REPORT THE ALLEGED CRIMINAL CONDUCT TO THE APPROPRIATE  
19 CRIMINAL JUSTICE AGENCY~~[-, INCLUDING WHETHER ANY STATUTORY REPORTING~~  
20 ~~REQUIREMENTS EXIST]~~. IF THE BOARD HAS A REASONABLE BELIEF THAT CONDUCT BY  
21 A LICENSED, PERMITTED OR CERTIFICATED INDIVIDUAL OR OTHER ENTITY OVER  
22 WHICH THE BOARD DOES NOT HAVE JURISDICTION MAY VIOLATE THE LAW OR CODES OF  
23 CONDUCT, THE BOARD SHALL REPORT THE CONDUCT TO THE APPROPRIATE STATE  
24 REGULATORY BOARD OR STATE AGENCY THAT THE BOARD REASONABLY BELIEVES HAS  
25 JURISDICTION OVER THE LICENSEE, PERMITTEE OR CERTIFICATE HOLDER OR OTHER  
26 ENTITY.

27 6. THE BOARD SHALL IMPLEMENT A POLICY PRIORITIZING COMPLAINTS BASED  
28 ON THE HARM TO A PATIENT OR POTENTIALLY TO THE PUBLIC. THE BOARD SHALL  
29 ASSIGN THE HIGHEST PRIORITY TO COMPLAINTS ALLEGING SEXUAL MISCONDUCT WITH  
30 A PATIENT, CRIMINAL ASSAULT OR THEFT OR PROVIDING SERVICES WHILE UNDER THE  
31 INFLUENCE OF ANY ILLEGAL OR LEGAL SUBSTANCE THAT IMPAIRS THE LICENSEE OR  
32 CERTIFICATE HOLDER.

33 7. THE BOARD SHALL PROVIDE THE RESPONDENT WITH A WRITTEN NOTICE  
34 [STATING] THAT THERE IS AN OPEN INVESTIGATION, [THE SUBSTANCE OF THE  
35 COMPLAINT.] THAT THE RESPONDENT HAS THE RIGHT TO BE REPRESENTED BY LEGAL  
36 COUNSEL AND THAT THE RESPONDENT HAS AT LEAST FIFTEEN BUSINESS DAYS AFTER  
37 RECEIVING THE WRITTEN NOTICE BEFORE THE BOARD REQUIRES A RESPONSE. THE  
38 WRITTEN NOTICE SHALL ALSO INFORM THE RESPONDENT THAT THE BOARD MAY USE ANY  
39 STATEMENT THE RESPONDENT MAKES AGAINST THE RESPONDENT. THE RESPONDENT MAY  
40 WAIVE THE FIFTEEN-DAY TIME FRAME BY WRITTEN COMMUNICATION.

41 8. IF THE BOARD DETERMINES THAT A PSYCHOLOGICAL, PSYCHIATRIC OR  
42 OTHER MEDICAL EVALUATION OF THE LICENSEE OR CERTIFICATE HOLDER IS  
43 ESSENTIAL FOR THE BOARD TO MAKE A DECISION REGARDING A COMPLAINT AND  
44 ORDERS THE LICENSEE OR CERTIFICATE HOLDER TO OBTAIN AN EVALUATION, AND THE  
45 LICENSEE OR CERTIFICATE HOLDER REQUESTS THAT THE EVALUATION BE MADE BY A  
46 PROFESSIONAL OTHER THAN THE PROFESSIONAL RECOMMENDED BY THE BOARD, THE  
47 BOARD OR ITS DESIGNEE MAY ACCEPT AND APPROVE AN EVALUATION FROM A



1 PROFESSIONAL WHO HAS THE CREDENTIALS, TRAINING[,] ~~[AND]~~ EXPERTISE ~~[AND]~~  
2 IMPARTIALITY] REQUIRED TO ADDRESS THE ISSUES THE BOARD HAS REQUESTED IN  
3 ITS ORDER. THE BOARD MAY NOT REQUIRE THE LICENSEE OR CERTIFICATE HOLDER  
4 TO BE EVALUATED ONLY BY A PROFESSIONAL WHOSE NAME IS PROVIDED IN A LIST BY  
5 THE BOARD TO THE LICENSEE OR CERTIFICATE HOLDER.

6 9. WITHIN ONE HUNDRED EIGHTY DAYS AFTER THE BOARD RECEIVES A  
7 COMPLAINT AGAINST A LICENSEE OR CERTIFICATE HOLDER, THE BOARD SHALL DO ONE  
8 OF THE FOLLOWING:

9 (a) SUBMIT THE INVESTIGATION FOR REVIEW.

10 (b) ADMINISTRATIVELY DISMISS THE COMPLAINT IF THE BOARD FINDS IT IS  
11 UNSUBSTANTIATED.

12 (c) REPORT A DETERMINATION EITHER THAT THE COMPLAINT INVESTIGATION  
13 CANNOT BE REASONABLY COMPLETED WITHIN ONE HUNDRED EIGHTY DAYS DUE TO THE  
14 COMPLEXITY OF THE MATTER[,] ~~[OR THAT]~~ THE ~~[RESPONDENT HAS REQUESTED]~~  
15 [RESPONDENT'S REQUEST FOR] ADDITIONAL TIME TO RESPOND OR ~~[HAS CAUSED]~~  
16 [CAUSING] DELAYS IN THE INVESTIGATION [OR THE FAILURE OF A PERSON OR  
17 ENTITY TO TIMELY RESPOND TO A BOARD SUBPOENA]. IF THIS DETERMINATION IS  
18 MADE, THE BOARD, WITHIN THE ONE HUNDRED EIGHTY-DAY PERIOD, SHALL TAKE ONE  
19 OF THE FOLLOWING ACTIONS:

20 (i) CONTINUE THE INVESTIGATION FOR AN ADDITIONAL ONE HUNDRED DAYS  
21 TO COMPLETE THE INVESTIGATION AND PROCEED WITH THE ADMINISTRATIVE  
22 PROCEDURE TO SUBMIT THE COMPLAINT FOR FINAL BOARD REVIEW AND ACTION.

23 (ii) ADMINISTRATIVELY DISMISS THE COMPLAINT WITHOUT PREJUDICE. IF  
24 THE BOARD ADMINISTRATIVELY DISMISSES THE COMPLAINT WITHOUT PREJUDICE, THE  
25 BOARD MAY REOPEN THE INVESTIGATION ONLY IF THE BOARD RECEIVES ADDITIONAL  
26 EVIDENCE, INFORMATION OR TESTIMONY SUFFICIENT TO CONCLUDE THE  
27 INVESTIGATION [WITHIN TWO YEARS AFTER THE ADMINISTRATIVE DISMISSAL AS  
28 DESCRIBED IN THIS ITEM].

29 10. IF ANOTHER HEALTH PROFESSION REGULATORY BOARD OR A CRIMINAL  
30 JUSTICE AGENCY IS INVESTIGATING THE CIRCUMSTANCES ALLEGED IN A COMPLAINT,  
31 THE TIME FRAMES PRESCRIBED IN PARAGRAPH 9 OF THIS SUBSECTION ARE SUSPENDED  
32 UNTIL THE INVESTIGATION IS COMPLETED.

33 B. FOR THE PURPOSES OF THIS SECTION, "WITHOUT PREJUDICE" MEANS THAT  
34 THE BOARD MAY OPEN ANOTHER COMPLAINT BASED ON THE SAME SET OF FACTS OF A  
35 COMPLAINT THAT HAS BEEN DISMISSED IF ADDITIONAL EVIDENCE OR INFORMATION  
36 BECOMES AVAILABLE TO SUBSTANTIATE THE COMPLAINT.

37 32-1664.02. Investigative files; access; confidentiality;  
38 definition

39 A. NOTWITHSTANDING ANY PROVISION OF THIS CHAPTER OR CHAPTER 32 OF  
40 THIS TITLE TO THE CONTRARY:

41 1. THE BOARD SHALL PROVIDE NOTICE TO A RESPONDENT AT LEAST FIFTEEN  
42 BUSINESS DAYS BEFORE A MEETING OF THE BOARD TO REVIEW THE STATUS OF AN  
43 INVESTIGATION. THE BOARD SHALL PROVIDE THE RESPONDENT WITH ACCESS TO THE  
44 INVESTIGATIVE FILE BY EITHER OF THE FOLLOWING METHODS AT THE DISCRETION OF  
45 THE RESPONDENT:

1 (a) ALLOW THE RESPONDENT TO REVIEW THE FILE AT THE BOARD OFFICE AND  
2 RECEIVE COPIES.

3 (b) PROVIDE THE FILE TO THE RESPONDENT OR THE RESPONDENT'S ATTORNEY  
4 BY ELECTRONIC TRANSMISSION.

5 2. A PERSON WHO OBTAINS INFORMATION FROM THE BOARD PURSUANT TO THIS  
6 SECTION MAY NOT RELEASE THE INFORMATION TO ANY OTHER PERSON OR ENTITY OR  
7 USE THE INFORMATION IN ANY PROCEEDING OR ACTION EXCEPT IN CONNECTION WITH  
8 THE BOARD'S REVIEW OF THE INVESTIGATION, THE DISCIPLINARY INTERVIEW AND  
9 ANY ADMINISTRATIVE PROCEEDING OR APPEAL RELATED TO THE DISCIPLINARY  
10 INTERVIEW OR HEARING. A PERSON WHO VIOLATES THIS PARAGRAPH COMMITS AN ACT  
11 OF UNPROFESSIONAL CONDUCT.

12 3. THE BOARD MAY NOT MAKE AN ADMINISTRATIVE DISMISSAL OR  
13 NONDISCIPLINARY REMEDIAL ACTION PUBLICLY AVAILABLE OR REPORT THE ACTION TO  
14 THE NATIONAL PRACTITIONER DATA BANK, CONSISTENT WITH FEDERAL LAW.

15 B. FOR THE PURPOSES OF THIS SECTION, "INVESTIGATIVE FILE" INCLUDES:

16 1. ANY WITNESS OR COMPLAINANT STATEMENT AND ANY DOCUMENTARY  
17 EVIDENCE OBTAINED DURING THE INVESTIGATION.

18 2. ANY REVIEW CONDUCTED BY AN EXPERT OR CONSULTANT PROVIDING AN  
19 EVALUATION OF OR OPINION ON THE ALLEGATIONS.

20 3. ANY RECORDS ON THE PATIENT OBTAINED BY THE BOARD FROM OTHER  
21 HEALTH CARE PROVIDERS.

22 4. THE RESULTS OF ANY EVALUATION OR TEST OF THE LICENSEE OR  
23 CERTIFICATE HOLDER CONDUCTED AT THE BOARD'S DIRECTION.

24 5. ANY OTHER FACTUAL INFORMATION THAT THE BOARD WILL USE IN MAKING  
25 ITS DETERMINATION.

26 6. ANY RECOMMENDATION TO THE BOARD FOR THE DISPOSITION OF THE  
27 INVESTIGATION.

28 32-1664.03. Burden of proof

29 EXCEPT FOR DISCIPLINARY MATTERS BROUGHT PURSUANT TO SECTION 32-1601,  
30 PARAGRAPH 27, SUBDIVISION (n), THE BOARD HAS THE BURDEN OF PROOF BY CLEAR  
31 AND CONVINCING EVIDENCE FOR DISCIPLINARY MATTERS BROUGHT PURSUANT TO THIS  
32 CHAPTER.

33 32-1664.04. Disciplinary actions; expungement; exceptions

34 NOTWITHSTANDING ANY OTHER PROVISION OF THIS CHAPTER TO THE CONTRARY:

35 1. THE BOARD MAY GRANT A REQUEST FOR EXPUNGEMENT OF A DISCIPLINARY  
36 ACTION PREVIOUSLY IMPOSED AGAINST A LICENSEE OR CERTIFICATE HOLDER,  
37 WHETHER FORMAL OR INFORMAL, ONLY AS AUTHORIZED BY THIS SECTION. THE BOARD  
38 MAY GRANT AN EXPUNGEMENT OF A DISCIPLINARY ACTION ONLY ONCE FOR A LICENSEE  
39 OR CERTIFICATE HOLDER.

40 2. ANY REQUEST FOR EXPUNGEMENT PURSUANT TO THIS SECTION SHALL BE  
41 MADE IN WRITING AND SHALL COMPLY WITH THIS SECTION AND ANY APPLICABLE  
42 RULES ADOPTED BY THE BOARD.

1        3. ANY DISCIPLINARY ACTION ARISING FROM THE FAILURE TO TIMELY RENEW  
2 A LICENSE OR CERTIFICATE, THE FAILURE TO COMPLETE CONTINUING EDUCATION OR  
3 A DOCUMENTATION ERROR SHALL BE EXPUNGED BY THE BOARD IF ALL OF THE  
4 FOLLOWING APPLY:

5        (a) THE DISCIPLINARY ACTION IS AT LEAST TWO YEARS OLD.

6        (b) THE TERMS OF THE DISCIPLINARY ACTION HAVE BEEN MET.

7        (c) THERE HAVE BEEN NO SUBSEQUENT VIOLATIONS OF ANY OTHER  
8 PROVISIONS OF THIS CHAPTER OR THE RULES ADOPTED PURSUANT TO THIS CHAPTER.

9        4. EXCEPT AS PROVIDED IN PARAGRAPH 5 OF THIS SECTION, FOR ANY OTHER  
10 DISCIPLINARY ACTION NOT IDENTIFIED IN PARAGRAPH 3 OF THIS SECTION, THE  
11 BOARD MAY EXPUNGE A DISCIPLINARY ACTION IF ALL OF THE FOLLOWING APPLY:

12        (a) THE DISCIPLINARY ACTION AT ISSUE IS AT LEAST FIVE YEARS OLD.

13        (b) THE TERMS OF THE DISCIPLINARY ACTION HAVE BEEN MET.

14        (c) THERE HAVE BEEN NO SUBSEQUENT VIOLATIONS OF ANY OTHER  
15 PROVISIONS OF THIS CHAPTER OR THE RULES ADOPTED PURSUANT TO THIS CHAPTER.

16        (d) THE LICENSEE OR CERTIFICATE HOLDER HAS HAD A TOTAL OF NOT MORE  
17 THAN TWO DISCIPLINARY ACTIONS IMPOSED~~[, INCLUDING THE DISCIPLINARY ACTION~~  
18 ~~FOR WHICH EXPUNGEMENT IS REQUESTED.]~~ BY THE BOARD OR ANY OTHER REGULATORY  
19 BOARD OR AUTHORITY FROM THIS STATE OR ANY OTHER STATE.

20        5. THE BOARD MAY NOT GRANT A REQUEST FOR EXPUNGEMENT IF ANY OF THE  
21 FOLLOWING APPLIES ~~[TO THE DISCIPLINARY ACTION FOR WHICH EXPUNGEMENT IS~~  
22 ~~REQUESTED]~~:

23        (a) THE DISCIPLINARY ACTION WAS BASED ON A CRIMINAL CONVICTION  
24 RESULTING FROM:

25        (i) ASSAULT, ABUSE, FRAUD OR ANY OTHER FELONIOUS CONDUCT THAT  
26 RESULTED IN PHYSICAL OR FINANCIAL HARM TO OR THE DEATH OF AN INDIVIDUAL.

27        (ii) ATTEMPTED MURDER, ASSAULT, ABUSE OR FRAUD.

28        (b) THE LICENSEE OR CERTIFICATE HOLDER ASSAULTED OR ABUSED A  
29 PATIENT.

30        (c) THE LICENSEE'S OR CERTIFICATE HOLDER'S CONDUCT CAUSED HARM OR  
31 DEATH TO A PATIENT.

32        ~~[(d) THE LICENSEE OR CERTIFICATE HOLDER COMMITTED SEXUAL MISCONDUCT~~  
33 ~~WITH A PATIENT.]~~

34        ~~(e) THE LICENSEE OR CERTIFICATE HOLDER WAS PRACTICING WHILE~~  
35 ~~IMPAIRED.]~~

36        ~~(f) THE LICENSEE OR CERTIFICATE HOLDER COMMITTED FRAUD OR FALSIFIED~~  
37 ~~RECORDS IN A HEALTH CARE SETTING.]~~

38        6. THE BOARD SHALL REPORT AN EXPUNGEMENT UNDER THIS SECTION TO ANY  
39 NATIONAL DATABASE TO WHICH THE BOARD PREVIOUSLY REPORTED THE DISCIPLINARY  
40 ACTION.

41        7. A LICENSEE OR CERTIFICATE HOLDER IS NOT REQUIRED TO REPORT AN  
42 EXPUNGED DISCIPLINARY ACTION ON ANY FUTURE APPLICATION FOR ISSUANCE OR  
43 RENEWAL OF A LICENSE, PERMIT OR CERTIFICATE TO ANY REGULATORY BOARD OR  
44 AGENCY IN THIS STATE.

1       32-1664.05. Board liability; damages  
2       NOTWITHSTANDING ANY OTHER LAW, IF THE BOARD DOES NOT COMPLY WITH  
3 THIS CHAPTER, A LICENSEE OR CERTIFICATE HOLDER OR THE SUBJECT OF A BOARD  
4 INVESTIGATION WHO WAS DAMAGED BY THE BOARD'S FAILURE TO COMPLY WITH THIS  
5 CHAPTER MAY SEEK DAMAGES, INCLUDING ATTORNEY FEES, IN A CIVIL LEGAL  
6 PROCEEDING. [A BOARD MEMBER OR STAFF MEMBER MAY BE HELD PERSONALLY LIABLE  
7 ONLY IF THE PERSON'S CONDUCT WAS RECKLESS, MALICIOUS OR WILFUL.]

8 Enroll and engross to conform  
9 Amend title to conform  
And, as so amended, it do pass

SELINA BLISS  
CHAIRMAN

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